

## Responding to childhood trauma

For much of the 20th century, the male arena of war was the focus for understanding traumatic experiences. Late-19th-century psychiatry had included the sphere of the family and childhood sexual abuse, but they were largely left aside until the 1980s. Since post-traumatic stress disorder (PTSD) was introduced as a DSM diagnosis in 1980, what counts as traumatic events and symptoms has been continually contested and modified. Childhood maltreatment is now considered a form of interpersonal trauma involving physical, sexual, or emotional abuse, or neglect, but understandings are still shifting.

One way to address trauma is through diagnoses. With roots in Judith Herman's 1990s work on prolonged exposure and complex trauma, and Bessel van der Kolk's early 2000s argument on developmental trauma, the new ICD-11 diagnosis of complex PTSD recognises that chronic or repeated trauma can have pervasive effects. In addition to the three PTSD symptom clusters (re-experiencing the event, avoiding traumatic reminders, and heightened sense of threat), complex PTSD involves three clusters related to self-organisation, including affect dysregulation, extremely negative self-concept, and difficulties in forming and maintaining relationships. This diagnosis is particularly relevant for people who experienced childhood maltreatment, and it helps to focus trauma-informed responses.

But diagnoses are challenging. Children with trauma can be misdiagnosed with bipolar disorder, psychotic disorders, or disruptive mood dysregulation disorder, and the new complex PTSD clusters overlap somewhat with those of borderline personality disorder. Traumatic disorders are also not the only relevant diagnoses. In one study, nearly one in three children who experienced trauma had a major depressive episode and one in four had self-harmed by age 18 years. Complicating the picture, a 2021 study found that experience of complex trauma with multiple interpersonal threats was associated with more severe psychopathology at age 18 than non-complex trauma. The effects are also physical; people with a history of childhood trauma are twice as likely to experience chronic pain in adulthood, and associations have been found with abdominal and lower back pain, irritable bowel syndrome, and migraine.

Beneath diagnoses, the mechanisms through which trauma takes effect can influence therapeutic responses.

Mariesa Cay and colleagues describe a neurobehavioural model of how childhood maltreatment leads to altered pain processing and psychopathology. They outline how dysregulation of threat detection and fear processing circuitry after childhood maltreatment can change perceptions of internal and external stimuli and subjective feelings, contributing to the development of chronic pain and anxiety, or conversely to dissociative symptoms in pain hyposensitivity. They also explore neurobiological reward processes, which are involved in affective aspects of pain processing and pain-induced mood states and analgesia; reward dysregulation after maltreatment is linked to depressive symptoms in adolescents. Given that the symptoms they focus on have been associated with trauma at different ages, they argue that symptoms rather than diagnoses could be a more precise avenue of understanding.

Individuals exist within social contexts, however, and Eamon McCrory and colleagues explore how altered neurocognitive functioning after maltreatment influences the interactive processes by which people construct their social architecture: their social status, social integration, and interpersonal stress. People with a history of maltreatment are more likely to experience other stressful social relationships (eg, peer rejection in adolescence), and childhood maltreatment contributes to social thinning, whereby a child's network of supportive relationships diminishes. McCrory and colleagues examine how altered threat processing could influence such changes, for example through hypervigilance towards, or avoidance of, threat cues leading to aggressive behaviour or withdrawal. Social networks thus gain greater significance for supporting young people's resilience.

Childhood trauma is common. Establishing diagnoses and mechanisms are one response to trauma, but mental health services should also focus on what happened to someone and how best to support them. For people of all ages, asking about trauma and its effects should be normalised. Trauma-informed, person-centred care should avoid people becoming retraumatised or feeling ignored, disrespected, or more afraid; patients need to feel safe and heard, have their strengths recognised, and be able to choose and control their care. ■ [The Lancet Psychiatry](#)



Simon Lee/Unplash

For more on **complex PTSD** see [Seminar Lancet 2022; 400: 60–72](#)

For more on **trauma and subsequent psychopathology** see [Articles Lancet Psychiatry 2019; 6: 247–56](#)

For the **contribution of complex trauma to psychopathology** see [BJPsych 2021; 219: 448–55](#)

For the **association between childhood trauma and chronic pain** see [J Pain 2016; 17 \(suppl 9\): T70–92](#)

For the **biobehavioural model of childhood maltreatment and the development of pain and psychopathology** see [Articles Lancet Child Adolesc Health 2022; 6: 95–106](#)

For more on **social thinning and stress generation after childhood maltreatment** see [Review page 828](#)